TCHP with Phesgo

Six 21-day cycles before surgery: Phesgo, docetaxel and carboplatin on Day 1. Surgery and pathology determine the postoperative HER2 medicine and remaining course.

Source reconciled; independent clinical, pharmacy and patient/PPI review pending. Selected effects; not a complete medicine leaflet.

A picture of side effects

Numbered symptom boxes connect to the anatomical illustration. The medicine responsible may be uncertain; report the symptom.

1 Heart / breathing

Phesgo

Keep echoes; report breathlessness, swelling or reduced exercise tolerance.

3 Injection reaction

Phesgo

Tell staff immediately about rash, chest tightness or breathing difficulty.

5 Hands / feet

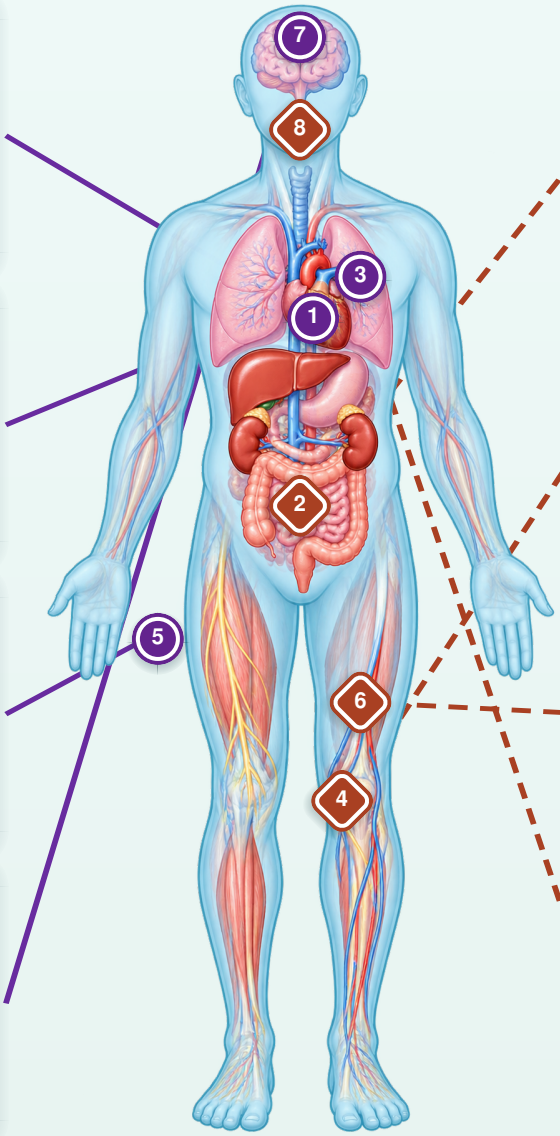
Docetaxel

Report numbness or tingling before the next dose.

7 Hair / scalp

Chemotherapy

Hair loss is common; ask about scalp cooling where suitable.



2 Diarrhoea

Especially pertuzumab

Report watery stools early, particularly with poor intake or dizziness.

4 Thigh injection site

Phesgo

Report persistent or spreading redness, swelling or pain.

6 Blood / infection

Chemotherapy

Fever, shivers or suddenly unwell: call immediately.

8 Mouth / appetite

Chemotherapy

Painful ulcers or reduced drinking need advice.

Selected effects, not every possible effect or a prediction. Blood-count, skin and line markers are schematic. Number and text carry meaning; colour is decorative.

The body map follows the medicines active in this phase.

> Fever, shivers or suddenly unwell: contact oncology immediately. Severe chest pain, breathing difficulty, collapse or seizure: 112 / 999.

Your treatment, at home.

Important medicine-specific effects

Docetaxel

- Low blood counts, infection and sepsis
- Hair loss, mouth soreness and fatigue
- Numbness / tingling; sometimes lasting
- Fluid retention and nail changes
- Allergic infusion reaction

Carboplatin

- Low blood counts, infection and bleeding
- Nausea, vomiting and fatigue
- Allergic reaction, sometimes after previous exposure
- Kidney or electrolyte changes

Pertuzumab

- Diarrhoea and dehydration
- Injection / allergic reaction
- Heart pumping-function reduction
- Rash and fatigue

Trastuzumab

- Heart pumping-function reduction / heart failure
- Injection or allergic reaction
- Diarrhoea and fatigue
- Rare serious lung effects

Surgery, radiation and monitoring: open the whole-treatment map or scan the QR.

Make the plan practical

Antiemetic plan

Carboplatin Day 1 uses its high-risk antiemetic plan. Docetaxel steroid premedication must be reconciled with antiemetic steroids.

Premedicines

Docetaxel: prescribed dexamethasone starts the day before each dose. Ask for one reconciled steroid schedule.

Appointments and results

Echo before treatment and every 12 weeks (around cycle 5 if on time), including maintenance. Bloods before each cycle. Ask who books and reviews each test; report a missing appointment.

Phase transition

After surgery, your team reviews the pathology and chooses the next HER2 medicine. The usual plan aims to complete about one year of HER2 treatment from the first dose. Remaining invasive cancer can change the medicine offered. The selected treatment has its own blood tests and supportive medicines.

Emergency: 112 / 999. Severe breathing difficulty, collapse, seizure or severe chest pain needs emergency help.

Contact oncology immediately for temperature ≥37.5°C or ≤36°C, shivering or suddenly feeling unwell, even with a normal temperature. Follow your own hospital's instructions. If you cannot reach the team, seek urgent medical assessment.

YOUR ONCOLOGY TEAM · DAYTIME

Insert numbers here

Acute Oncology Service

YOUR ONCOLOGY TEAM · OUT OF HOURS

Insert numbers here

Ask for medical oncology

EVENINGS / OVERNIGHT

Your out-of-hours pathway / nearest ED

Other hospitals: use the numbers your team provides.

Complete reference, visual timeline and passport

https://sactcheck.com/patient/00789/

Sources and review status are available online. Follow your own prescription and hospital instructions.